

February 23, 2026

Thomas Keane, MD, MBA
Assistant Secretary for Technology Policy
Office of the National Coordinator for Health Information Technology
Department of Health and Human Services
Attention: HHS Health Sector AI RFI

RIN: 0955-AA13

SUBMITTED ELECTRONICALLY ([HTTP://WWW.REGULATIONS.GOV](http://www.regulations.gov))

Dear Dr. Keane:

We write to the Department of Health and Human Services (“HHS”) on behalf of a digital health and medical device client. We appreciate the opportunity to submit comments on the Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care.

Introduction

On the eve of the first internet era, lawmakers recognized the necessity of ensuring the free flow of information within the health care industry. Congress passed the Health Insurance Portability and Accountability Act (“HIPAA”) in 1996 to accomplish three goals: ensure continuity of individual insurance coverage, protect patient agency with respect to their protected health information (“PHI”), and streamline the electronic exchange of health information.¹ By emphasizing portability of care, coverage, and data, HIPAA laid the groundwork for a more efficient and interconnected health care ecosystem.

Thirty years later, we are in the midst of another remarkable period of technological innovation spurred by artificial intelligence and machine learning technologies. But decisive regulatory action is once again necessary to ensure the free flow of information. Doing so will continue to catalyze

¹ See William J. Clinton, *Statement on Signing the Health Insurance Portability and Accountability Act of 1996*, Online by Gerhard Peters and John T. Woolley, The American Presidency Project <https://www.presidency.ucsb.edu/node/222651> (Aug. 21, 1996).

a diverse and competitive marketplace where AI entrepreneurship is rewarded and better health outcomes are delivered.

Today, certain industry actors are seeking to consolidate power and ownership of health data by exploiting the rules, policies, and terms of access to Health Information Exchanges (“HIEs”) and Qualified Health Information Networks (“QHINs”). Tools clearly meant to promote information sharing are being used to lock out digital health and medical device companies on the baseless grounds that these entities cannot provide “treatment” or “health care” as understood under HIPAA, with the implicit connotation that treatment is the exclusive domain of hospitals and physician practices, and their electronic health record vendors. This rationale completely rewrites HIPAA and the 21st Century Cures Act and ignores the policy goals of each core piece of legislation.

Data is the lifeblood of AI innovation. Health data access should not be something that plays out in private interest business litigation, closed-door arbitrations, or dense pages of HIE participation rules and agreements—it belongs in the hands of the patient. Indeed, HHS recognizes that patients should be able to “access their electronic medical information anywhere that it lives on the network” and that health apps should “not require any special status to be on the network.”² Moreover, such access should occur “seamlessly,” without requiring a patient to jump through unnecessary hoops or “repeatedly recall and write out their medical history.”³ Restricting digital health and medical device companies’ access to HIEs not only stifles AI advancement and disregards the key role these companies play in patients’ day-to-day lives; it eschews bedrock principles of patient access and autonomy.

Accordingly, HHS should protect digital health and medical device companies’ ability to participate in HIEs by requiring QHINs to admit to HIEs those companies that directly interact with patients and provide “treatment” as that term is and has been long understood under HIPAA. This can and should permit a wide variety of actors to participate in the health data community in order to execute HHS’s salient policy goals of patient access and empowerment, accelerating the use of AI in health care, and reducing red tape and anticompetitive barriers to marketplace innovation.

I. Regulatory Background

HIPAA

With the passage of HIPAA, Congress directed HHS to standardize the format for the exchange of health information. Standardization improves the accuracy and reliability of health information, reduces administrative overhead and cost associated with competing transmission formats, and enhances the coordination of care between health care entities.⁴ More than just cost savings, HHS

² Interoperability Framework, CTRS. FOR MEDICARE & MEDICAID SERVS., <https://www.cms.gov/health-technology-ecosystem/interoperability-framework> (last modified July 31, 2025).

³ Patient Facing Apps – Kill the Clipboard, CTRS. FOR MEDICARE & MEDICAID SERVS., <https://www.cms.gov/health-tech-ecosystem/early-adopters/kill-the-clipboard> (last modified February 17, 2026).

⁴ 67 Fed. Reg. 53182, 53182 (Aug. 14, 2002).

recognized that accurate, efficient, and effective data exchanges drive better health outcomes for patients and improve the delivery of care.⁵

Moreover, an effective data exchange would meet the needs of a health data community that was no longer a closed, “one-to-one interaction[] between patients and clinicians, [but rather an] integrated health care delivery network”⁶ HHS understood that any data exchange must encourage and grow with a diverse community of health care actors. Specifically, the standards for data exchange would need to “incorporate flexibility to adapt more easily to changes in the health care infrastructure (such as new services, organizations, and provider types) and information technology.”⁷

These themes of innovation and interconnectivity permeate HIPAA. HHS emphasizes that whether an organization is a HIPAA-covered entity is “based on activities and functions” and “not titles.”⁸ Reinforcing this position, HHS identifies several types of non-traditional entities, including “pharmaceutical manufacturers” and “online companies” that may qualify as covered entities, provided that they meet the statutory definition of “health care provider” and furnish, bill, or are paid for “health care.”⁹ HHS, in February 2004, further illustrated this role-based, flexible standard by explicitly recognizing that medical device companies may receive PHI for treatment purposes.¹⁰ HHS acknowledges that these core HIPAA definitions cannot serve to gatekeep data and stymie progress, but rather must be firmly rooted in function in order to accommodate technological innovation.¹¹

TEFCA

Created under the 21st Century Cures Act, the Trusted Exchange Framework and Common Agreement (“TEFCA”) built upon HIPAA’s data exchange framework by establishing a set of standards issued by the Assistant Secretary for Technology Policy/Office of the National Coordinator for Health Information Technology to promote health data interoperability. This public-private initiative established the HIEs and the supporting QHINs to realize the policy goal

⁵ 65 Fed. Reg. 82462, 82465 (Dec. 28, 2000) (“Use of electronic information has helped to speed the delivery of effective care and the processing of billions of dollars worth of health care claims. Greater use of electronic data has also increased our ability to identify and treat those who are at risk for disease, conduct vital research, detect fraud and abuse, and measure and improve the quality of care delivered in the U.S.”).

⁶ *Id.* at 82465.

⁷ 63 Fed. Reg. 43242, 43244 (Aug. 12, 1998).

⁸ 65 Fed. Reg. at 82568.

⁹ *Id.* at 82574.

¹⁰ HHS FAQs, “When may a covered health care provider disclose protected health information, without an authorization or business associate agreement, to a medical device company representative?” (last updated Aug. 8, 2005), available at: <https://www.hhs.gov/hipaa/for-professionals/faq/490/when-may-a-covered-health-care-provider-disclose-protected-health-information-without-authorization/index.html>; HHS and CardioNet, Inc. Resolution Agreement, HHS (April 3, 2017), available at: <https://www.hhs.gov/sites/default/files/cardionet-ra-cap.pdf> (finding that a cardiac monitoring device company was a covered entity).

¹¹ See 63 Fed. Reg. 25320, 25321, 25325 (May 7, 1998).

of health data interoperability and continue to promote underlying objectives of patient care and choice.¹²

Health care providers, covered entities, and health IT developers with a valid purpose may access PHI via TEFCA. These purposes are treatment, payment, health care operations, public health, government benefits determinations, and individual access services. Entities *must* share data for treatment purposes but *may* share data for other purposes, like payment or health care operations. This dynamic echoes HIPAA's long-standing protection of the free flow of information for covered entities' treatment purposes.¹³

TEFCA SOP Changes

The Common Agreement for Nationwide Health Information Interoperability ("Common Agreement") governs data access to HIEs. TEFCA, through its Recognized Coordinating Entity ("RCE"), and QHINs are signatories to the Common Agreement, but the Common Agreement includes flow-down terms such that all HIE participants must agree to substantially the same language. The Common Agreement contemplates change management provisions and differentiates between changes to the Common Agreement and to Standard Operating Procedures, or those rules that govern participants' access to the QHINs.¹⁴

In July 2024, TEFCA released *Standard Operating Procedure (SOP): Exchange Purpose (XP) Implementation: Treatment* ("Treatment SOP"). The Treatment SOP created a new subcategory of HIE data access requests, or queries, entitled "TEFCA Required Treatment." HIE participants must respond to treatment queries sent under this exchange purpose code. Conversely, treatment queries that fall under the generic treatment code are permissive requests for information, and thus, optional.¹⁵ The Treatment SOP states that only certain covered entities may initiate transactions

¹² See, e.g., *ONC's Cures Act Final Rule*, HealthIT.gov, <https://www.healthit.gov/topic/oncs-cures-act-final-rule> (last updated March 26, 2024) ("The rule is designed to give patients and their healthcare providers secure access to health information. It also aims to increase innovation and competition by fostering an ecosystem of new applications to provide patients with more choices in their healthcare").

¹³ See 45 C.F.R. § 164.506(c). See also 45 C.F.R. § 164.502(b)(2)(i) (excluding treatment activities from HIPAA's "minimum necessary" requirement); 65 Fed. Reg. at 82493 (expressing clear intent not to interfere with patient treatment by promulgating an exception to uses and disclosures of PHI requiring patient authorization).

¹⁴ Changes to the Common Agreement are subject to a procedural process outlined in the Common Agreement. While the RCE may elect to consult with various governing bodies within the HIE/QHIN structure regarding changes to the Common Agreement, it is not required to consult with the "Participant/Subparticipant Caucus." Further, proposed changes to the Common Agreement need only be ratified by a two-thirds' vote cast by QHIN caucus members. Similarly, for any changes to the SOPs, the RCE may elect to consult with various governing bodies within the HIE/QHIN structure, but it is not required to consult with the Participant/Subparticipant Caucus. Any material changes to the SOPs must be approved by at least two-thirds' vote of the QHIN caucus and Participant/Subparticipant Caucus. However, any non-material changes to SOPs need only be approved by two-thirds' vote of the QHIN caucus.

¹⁵ See *Standard Operating Procedure (SOP): Exchange Purpose (XP) Implementation: Treatment*, TEFCA at § 5.4.2.2 ("QHIN Query Response – All Responding Nodes MUST respond to QHIN Queries for the T-TRTMNT XP Code that contain the information specified in this SOP, in accordance with the Framework Agreements and Applicable Law.") compared to *id.* at § 4.4.2.2 ("QHIN Query Response – All Responding Nodes SHOULD Respond to QHIN Query Requests for the T-TREAT XP Code that contain the information specified in this SOP, in accordance with the Framework Agreements and Applicable Law."), available here: https://rce.sequoiaproject.org/wp-content/uploads/2024/07/SOP-Treatment-XP-Implementation_508.pdf.

under the TEFCA Required Treatment code. Specifically, only covered entities that are *licensed* health care providers—or their delegates, which includes their electronic health record vendors—may send TEFCA Required Treatment requests.¹⁶ TEFCA announced that these changes, which were described as ‘clarifying’ procedural changes, were “intentionally narrow” and issued by TEFCA in order to “build trust” in the TEFCA community.

Contrary to TEFCA’s trust-building, procedural classification, these changes created a two-tiered system in which certain covered entities were guaranteed access to health data and other covered entities were not.¹⁷ This structure significantly departs from the long-standing HIPAA principles that recognize and protect all health care providers’ ability to use and disclose PHI for treatment purposes. Moreover, the Treatment SOP was foisted upon the health data community without any discussion or debate. The changes were announced without the benefit of any meaningful procedural protections contemplated under the Common Agreement and fall far short of notice-and-comment rulemaking standards under the Administrative Procedure Act.

II. The Role of Digital Health and Medical Device Companies

Digital health and medical device companies are reshaping the health care industry by prioritizing a user-first philosophy that reimagines what it means to deliver patient-centered care. Unlike traditional health care systems that operate on rigid, appointment-based workflows where providers may interact with patients a few times a year at most, digital health and medical device companies leverage sleek, intuitive interfaces to incorporate health care into a patient’s day-to-day life. By focusing heavily on user interaction and high-fidelity design, these companies significantly reduce barriers to entry and engagement. Such low-friction touchpoints foster a richer and more textured relationship with the patient, transforming health from an episodic chore into a continuous, proactive dialogue.

A high level of engagement promotes better data collection, and allows these companies to capture a broader spectrum of real-world data than typical hospitals and health care systems. Through the integration of wearable devices, IoT-enabled diagnostic tools, and other health related mobile apps, these platforms gather longitudinal data on key health metrics like sleep patterns, heart rate variability, physical activity, ovulation cycles, wellness trackers, and nutritional intake. This data supports not only healthy habits in general, but promotes key health drivers and objectives, like chronic condition counseling, medication adherence, care coordination and navigation, remote patient monitoring, post-treatment therapies, and other care counseling services. When this data is supplemented with data obtained in more traditional health care settings and processed using digital health and medical device companies’ sophisticated, AI-driven data analytics, hyper-personalized care interventions become possible. For example, an app may nudge a diabetic patient to adjust their nutritional intake based on immediate glucose trends or identify early signs of mental health decline through behavioral changes and daily journaling practices.

Digital health and medical device companies are uniquely positioned to bridge the care gap that exists, not only from check-up-to-check-up, but between the patient in the clinical context and the

¹⁶ See *id.* at § 5.3. Government organizations like the Veterans Health Administration may also send requests under the TEFCA Required Treatment code.

¹⁷ See *id.* at pg. 3.

patient in their real-world setting. On-demand and multifaceted engagement surfaces—such as automated data collection, daily reminders and push notifications, 24/7 chat support, or instant virtual consultations—reorient the delivery of care around the patient, wherever they may be. This combination of high-definition data, sophisticated analytical offerings, and human-centric design demonstrates the unique value that digital health and medical device companies provide to the health care ecosystem.

Unfortunately, certain industry actors interpret this value proposition as a threat instead of an opportunity to collaborate and achieve better health outcomes. Permitting only some members of the health data community to meaningfully partake in the HIEs will stymie the innovative and competitive marketplace goals that HHS seeks to foster. Exclusion (on the basis of an inaccurate reading of HIPAA and closed-door drafting sessions) creates a two-tiered system for entities—those that can participate in legitimate business activities and those that cannot—and ultimately reduces patient choice and autonomy in the marketplace.

III. Responses to Specific Requests for Information

a. Response to RFI Question No. 3

Question No. 3: For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

Certain industry actors are seeking to misappropriate HIPAA’s definitions of “treatment,” “covered entity,” and/or “health care” in the Common Agreement, and the supporting terms of HIE access, to block digital health and medical device companies from participating in the HIEs. At best, these actors concede that relevant HHS guidance creates ambiguity and gray areas as to which entities may be capable of performing “treatment” or what “treatment” may look like in practice. On the contrary, as described in more detail above, long-standing guidance from HHS’ rulemaking activity, including a piece of guidance that is over twenty years old, explicitly recognizes the variety of actors that may indeed provide “treatment” and thus are permitted to access PHI for such purposes.

HHS should recognize and reinforce its flexible definition of “treatment” under HIPAA by issuing new guidance that expressly acknowledges the role that direct-to-patient, AI-powered services can and do play in today’s health ecosystem—including providing and supporting “treatment” purposes.

b. Response to RFI Question Nos. 2, 5

Question No. 2: What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

Question No. 5: How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

HHS should continue to support private sector AI innovation by providing an accreditation or certification for digital health and medical device companies that provide certain innovative health services powered by AI. Such an accreditation or certification would serve as an additional means to substantiate these entities' "treatment" purpose, for purposes of meeting TEFCA's permitted purposes access requirements and any corresponding implementation instructions or guidelines. This alternative path would provide a neutral setting where digital health and medical device companies could demonstrate their clinical value, instead of fighting against well-funded, self-interested parties that are currently commandeering the HIE/QHIN rules of engagement.

HHS should also consider (i) amending the Common Agreement to prohibit non-discrimination of digital health and medical device companies that have direct interactions with patients; (ii) requiring *any* changes to HIE or QHIN participation policies, business rules, or other applicable standards governing access to health data be subject to a vote involving all QHIN participants and subparticipants—not just QHIN caucus members;¹⁸ and (iii) immediately unwinding the two-tiered system established under the Treatment SOP. When private actors arbitrarily and capriciously change or enforce such terms to exclude certain parties, not only is the result unfair and anticompetitive, but it threatens the health and safety of patients who incorporate those digital health technologies in their day-to-day health care.

c. Response to RFI Question No. 8

Question No. 8: Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

AI technologies are only as good as the data on which they are based. Effective AI development requires data from a variety of contexts and encounters across the patient's health care journey. This comment seeks to underscore the necessary condition precedent—that digital health and medical device companies are permitted HIE data access in the first place.

Conclusion

HHS has long recognized that health care is not limited to the four corners of a doctor's office. A thriving health data ecosystem pushes actors to meet patients where they are—online, in connected devices, or in the palm of the patient's hand. To best realize HHS' goal of AI innovation and adoption, HHS must act to protect digital health and medical device companies' ability to access HIE data. QHINs must admit digital health and medical device companies that directly interact with patients and recognize that non-traditional actors must access this data for "treatment" purposes. Preserving this free market balance will promote patient agency and better health outcomes and help establish the U.S. as a global leader in AI.

¹⁸ See § 5.3.4 of the Common Agreement.

We appreciate the chance to provide HHS with information about the industry and comments to your requests for information that we presented in this comment letter. Please do not hesitate to contact us if you have any questions.

Sincerely,

A handwritten signature in blue ink, consisting of a series of loops and a long horizontal stroke extending to the right.

Adam C. Solander